

PATIENT: WHITFIELD, DANA L. DOB: 03/14/1991 MRN: SIC-77320
EXAM: MRI BRAIN W/WO CONTRAST (CPT 70551) + MRV HEAD DATE: 12/02/2022
REFERRER: P. NATARAJAN, MD · Springfield Neurology Associates

CLINICAL HISTORY

31-year-old female with two months of progressive headaches and transient visual obscurations. Fundoscopic exam 11/21/2022 documented bilateral papilledema. Patient denies pulsatile tinnitus. Presents without vision loss at time of exam. Evaluate for secondary causes of intracranial hypertension prior to lumbar puncture.

TECHNIQUE

Multiplanar multisequence MRI brain performed before and after 7 mL gadobutrol. 2D time-of-flight MRV of the head performed without contrast.

FINDINGS

Brain parenchyma: No acute infarct, hemorrhage, mass, or mass effect.

Ventricles: Normal in size and configuration. No hydrocephalus.

Sella: Partially empty sella noted.

Orbits: Mild flattening of the posterior globes bilaterally with distension and tortuosity of the optic nerve sheath complexes, findings that can be seen with elevated intracranial pressure.

Venous system (MRV): The dural venous sinuses are patent. No evidence of dural venous sinus thrombosis or focal high-grade stenosis.

IMPRESSION

1. No intracranial mass, hemorrhage, or venous sinus thrombosis.
2. Partially empty sella, posterior globe flattening, and optic nerve sheath distension: constellation compatible with idiopathic intracranial hypertension in the appropriate clinical setting.
3. Recommend correlation with lumbar puncture opening pressure.

ELECTRONICALLY SIGNED

R. Vasquez, MD · Neuroradiology

Signed: 12/02/2022 16:41

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